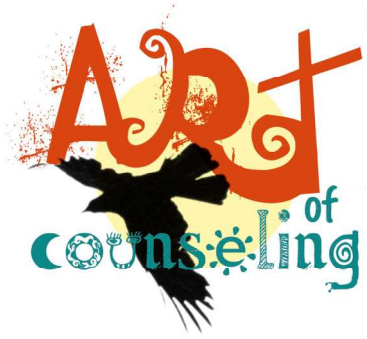




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June 15, 2026

Dr. Marie Claire Buckley
M Health Fairview Plastic and Reconstructive Surgery Clinic Minneapolis
909 Fulton Street Southeast, Floor 4, Minneapolis, MN 55455

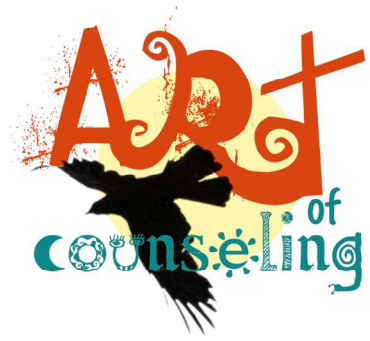
Re: Nathalia Araujo (patient name on insurance card)
Emilio Romero (patient chosen name)
August 3, 2000 (patient DOB)

Dear Dr. Marie Claire Buckley:

Nathalia Araujo (legal name) is a patient in my care at Art of Counseling, pllc. I have a Master's degree (MA) in Counseling and Psychotherapy from St. Cloud State University Graduate School and am licensed as a Licensed Professional Clinical Counselor (LPCC) by the state of Minnesota. As such, I have competence in using the current version of the Diagnostic Statistical Manual of Mental Disorders (DSM), including the ability to recognize and diagnose coexisting mental health concerns and distinguish these from gender dysphoria. I am knowledgeable about gender nonconforming identities and expressions, along with the evaluation and treatment of gender dysphoria. I have received specific continuing education in the assessment and treatment of gender dysphoria.

Nathalia/Emilio has been a patient of mine since September 25, 2025, during which we have met on a weekly basis for a total of 29 hours of psychotherapy. Nathalia/Emilio identifies as male and goes by He/Him. Nathalia/Emilio notes that they first knew their gender identity differed from their assigned sex at about age 12. Client reports, "I was around 12 years old when I first felt off about my assigned sex. The breakthrough was when I was a freshman in high school (age 13) where I started feeling gradually uncomfortable being referred as a cis woman. Initially, it was to cope with trauma but it ended up becoming a part of my identity to be a trans man after my sophomore year."

Additional gender history information reported by the client is, "I've been on Testosterone (initially Cypionate injected inter-muscular) since August 2022 but I felt incredibly marginalized due to anti-trans laws, including those that limited trans-healthcare, while I lived in Florida. I was nearly 1 year on treatment before the laws came into fruition and it caused me to become a complete shut-in and turn paranoid because I was afraid of being killed for being openly transgender in Florida (there were trans hate crimes happening near where I lived at the time). I moved to Minneapolis for that main reason: to feel safer in terms of legal protections regarding my identity and to continue my medical transition as I was forced to stop (had even planned to DIY testosterone through my mom's friend that lived in South America)."



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They have been successfully and consistently living in a gender role congruent with their affirmed gender for about 12 years since about 2014. Client reports, “My family was unsure about it initially and said it was a phase until I started being incredibly persistent with them about my name, gender, and pronouns. They came to accept my identity around 2016. I have been comfortable with my current gender identity ever since.”

They have socially transitioned by changing their name, pronouns, and coming out to family, friends, and at work. Client reports, “I always tell everyone I meet my preferred name, pronouns, what my gender identity is. I do not like having to hide who I am.”

Nathalia/Emilio reports for hormone therapy use, “I am on HRT (Testosterone). I plan to continue being on it for the rest of my life as needed. HRT helps me feel more human, more authentic to the real me. I plan to take the next steps to have top surgery in the near future.” Client is looking forward to having a consultation soon for top surgery.

Despite these interventions, the client reports significant anxiety, depression, and distress due to their persistent experience of dysphoria, as well as continuing to be misgendered by others. Client reports, “I used to feel significant dysphoria when I was younger to the point of self-harm. Nowadays, it does still bother me but I tell myself all the time “they don’t know who I am and that’s okay”. I feel more euphoric nowadays knowing I shave my facial hair (happy that I can even grow any), my voice is deeper, and I am usually seen as male due to how I dress! But the dysphoria is still existing.”

By my independent evaluation (i.e. diagnostic assessment) of Nathalia/Emilio, I have diagnosed them with Gender Dysphoria (ICD-10 F64.1). Nathalia/Emilio has expressed a persistent desire for a mastectomy/top masculinization surgery.

Nathalia/Emilio reports his goals of surgery are:

- 1) “To feel more comfortable going outside in public”
- 2) “To not have to worry about people perceiving me as a woman due to having breasts.”
- 3) “Wanting to feel more masculine.”

Nathalia/Emilio reports this will relieve his dysphoria in the following ways:

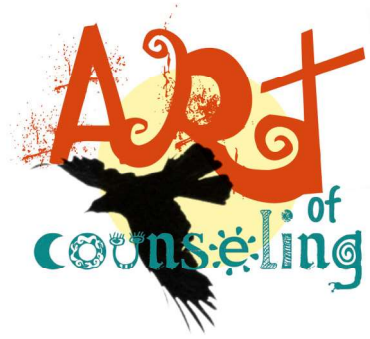
- 1) “By presenting more masculine in social spaces.”
- 2) “To feel more comfortable looking at myself in the mirror (basically being able to look at myself in general).”

In addition to gender dysphoria, I have diagnosed Nathalia/Emilio with;

- 1) Posttraumatic Stress Disorder
- 2) Major depressive disorder, Recurrent episode, Moderate
- 3) Generalized Anxiety Disorder

All of the listed diagnoses are well-managed through weekly psychotherapy sessions with me and ongoing medication management with the client’s psychiatrist.

The client’s current medications (as reported by the client) include,



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- 1) Inderal 10mg as needed twice daily
- 2) Abilify 5mg one before bed
- 3) Testosterone (1.62% transdermal gel) 4 pumps daily

The client reports his surgical history includes: “Tonsils removed when I was very young, I have no memory of before or after the procedure. Wisdom teeth removed March 2024.”

The client is stably housed in university dorm apartments. Client is working on plans and preparations for post-op recovery. The client reports options include, help from family who live in Brooklyn Center, MN and client’s mother flying to MN to help with care. Client reports he is still solidifying plans.

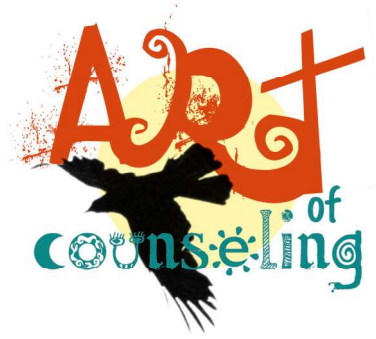
They have no issues with illicit drug use or abuse. Client reports, “I have no substance abuse issues. I only rarely use edibles (recreationally) in social situations. I have no reliance on cannabis.”

Nathalia/Emilio has more than met the WPATH criteria for a mastectomy/top masculinization surgery. I believe they have an excellent understanding of the risks, benefits, and alternatives to this surgery. They are capable of making an informed decision about undertaking surgery. I believe that the next appropriate step for them is to undergo a mastectomy/top masculinization surgery, and I believe this will help them make significant progress in further treating their gender dysphoria. Therefore, I hereby recommend and refer Nathalia/Emilio to have this surgery. I am available for coordination of care preoperatively and postoperatively. If you have any questions or concerns, please do not hesitate to contact myself or my office.

Sincerely,

Kathy Pascale, MS, LPCC
Pronouns: She/Her
Licensed Professional Clinical Counselor

kathy@artofcounselingstpaul.com
Direct Phone: 952-522-8318



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